

Classification and Intended Purpose Rubric

NHS Pathways Probabilistic Triage

Document control

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Revision history

Version	Date	Summary of changes	Revised by
0.1	TBC	Initial draft.	Martin Hodgson

Reviewers

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Related documents

Document ID	Title
PT-2.1	Intended Use Statement (declared Class I device)
PT-2.1-A	Platform Functionality Set Register and Intended Purpose by Set. Holds the per-function determination outcomes referred to in Section 1.
CP-2.1	Core Pathways Intended Use Statement
NHSP-10.3	Clinical Evaluation Plan and Clinical Evaluation Report
NHSP-10.4	Post-Market Surveillance Plan
MD-QMS-SOP-D001	Software Design Control
MD-QMS-SOP-D002	Medical Device Risk Management

MD-QMS-SOP-D007	Software Change Management
MD-QMS-SOP-D011	Labelling and Information for Users
MD-QMS-SOP-D017	Usability Engineering
MD-QMS-WI-D002	Software Safety Classification
MD-QMS-SOP-A002	Control of Document Changes
MD-QMS-SOP-B002	Medical Device Programme Terms of Reference
MD-QMS-SOP-D013	Clinical Evaluation
TBC	Risk Management File (ISO 14971)
TBC	Usability and human factors engineering file (IEC 62366-1)
TBC	Clinical Safety Case Report (DCB0129)
TBC	NHS Pathways host system accreditation requirements

1. Purpose and scope

Software carries no intended purpose in its physical form. The same probabilistic model that returns a care disposition can be Class I or Class IIa depending on what it is declared to do, for whom, and what it places in front of them. Classification begins with the intended purpose.

Probabilistic Triage is a platform carrying software functions with different regulatory positions, from operational services that are not medical devices to condition-level inference that would be Class IIa.

This rubric defines how each function is classified, how its intended purpose is stated, what separates it from its neighbours, and what happens to all three when any of them changes. It applies to every function, in every configuration in which it may be placed on the market, to every output, and to every proposed change to clinical logic, output set, user population, deployment channel or component boundary. The determination steps and the gate are function-agnostic: they apply to every function this platform carries over its life, not only those built today. The outcome for each function in scope now is recorded separately, outside this rubric; that record changes as functions are added, removed or modified, the rules that produced it do not.

2. How to use this document

Apply the classification steps in sequence. Each step builds on the previous result. Where a function spans multiple steps (for example, a feature that both generates a score and sends an automated patient notification), classify the entire function at the highest applicable classification across all its outputs. Group codes G0 to G4 are defined at Step 2, separation criteria S1 to S3 at 4.2, and contamination controls C1 to C4 at 4.3.

Abbreviations used in this document:

Abbreviation	Meaning
CAB	Change Advisory Board, constituted under MD-QMS-SOP-D007
CEP / CER	Clinical Evaluation Plan / Clinical Evaluation Report
CSO	Clinical Safety Officer, DCB0129

DoS	Directory of Services
G0 to G4	Function groups, defined at Step 2
C1 to C4	Contamination controls, defined at 4.3
S1 to S3	Separation test criteria, defined at 4.2
Management Representative	Regulatory Affairs and Quality Compliance Manager, the QMS approval authority
MDP	Medical Device Programme, the oversight board constituted under MD-QMS-SOP-B002
RA	Regulatory Affairs

3. The determination steps, applied per function

Step 1: Qualification

Question	Answer	Outcome
Q1. Does the function act on data relating to an identified or identifiable individual?	No	Not a device on this ground. Classify G0, go to Step 5a.
	Yes	Go to Q2.
Q2. Is there a medical purpose within regulation 2(1): diagnosis, prevention, monitoring, treatment or alleviation of disease; diagnosis, monitoring, treatment, alleviation of or compensation for an injury or handicap; investigation, replacement or modification of the anatomy or of a physiological process; or control of conception?	No	Not a device. A function may process identifiable data and still exit here, as an operational or infrastructure function typically does. Classify G0, apply boundary controls, go to Step 5a.
	Yes	Go to Q3.
Q3. Does the function create new information from that data, or only present, store or transmit what it received?	Presents, stores or transmits only	Not a device. The function is a conduit. Classify G0, apply boundary controls, go to Step 5a.
	Creates new information	Standalone software qualifying as a medical device, and an active device. Go to Step 2.

The definition of medical device in the 2002 Regulations expressly includes software; the word is not omitted when the definition is quoted. Qualification is settled for the core triage function and is not reopened at each change. Step 1 governs new functions and adjacent functionality.

Step 2: Function group (G0 to G4)

Assign the group that matches the function's highest output. Where a function emits several outputs, assign the highest applicable group.

Code	Group	Distinguishing criterion	UK MDR
G0	Non-device	Established at Step 1: no output states anything about an individual,	Not a device

		or the output is an unmodified pass-through of what was received.	
G1	Navigation	A route to care, or care parameters attached to it: timeframe, skill level required, safety-netting, self-care advice. Makes no statement about the individual's condition.	Class I, Rule 12
G2	Confidence-scored navigation	As G1, plus a displayed confidence or probability value in the route to care itself, not in a condition.	Class I, Rule 12.
G3	Condition-level inference informing a diagnosis	The output carries condition-level information: a differential, a named candidate condition, a probability attached to a condition, or content a reader uses diagnostically. A human reaches the diagnosis, but the device supplies the material they reach it from. Engaged whether or not the information is displayed. The classification boundary of the platform.	Class IIa, Rule 10 direct diagnosis limb. Influence alone is not automatically direct diagnosis. It becomes direct diagnosis where nothing between the output and the decision supplies an independent clinical judgement, which is the position on this platform: there is no routine clinical review of each output. Where a competent clinician does exercise independent judgement on every output, the position is reasoned on the determination record rather than assumed.
G4	Diagnosis	The output is the diagnosis. The device states, or delivers to an individual, what the condition is or is likely to be, and nothing between the output and the person acting on it supplies an independent clinical judgement. An indicative diagnosis presented to a lay user is a diagnosis for this purpose.	Class IIa, Rule 10 direct diagnosis limb.

G3 and G4 carry the same Annex IX class. The ladder separates them because the obligations do not match: at G3 the safety argument rests on the competence and independence of the reader, and that has to be evidenced; at G4 there is no reader to rest it on, and the device's own diagnostic performance carries the whole argument. Recorded as one group, a device could move from the first position to the second with no change of class to trigger a reassessment.

Autonomous clinical action is not a group on this ladder. The ladder classifies diagnostic output, and committing an action is a different question. A function that commits a clinical action itself engages Rule 9 where it administers or exchanges energy, implementing rule 2.3 where it drives or influences another active device, and otherwise falls to Rule 12 at Class I. It is held outside the device by boundary condition and escalation trigger, not by class, and it carries its IEC 62304 class and residual risk whatever the Annex IX outcome.

A displayed confidence value carries its own evidence obligations at G2, whatever the class: calibration evidence in the CER, over-reliance and under-reliance treated as distinct failure modes, and summative usability evaluation of the displayed value for every group that sees it.

Where a function's declared purpose includes an acuity-assessment claim, identifying presentations that may require an emergency or urgent response, that claim carries the Essential Requirement 3 and Annex X consequence in full: under-triage and over-triage need acceptance criteria in the CEP, evidence in the CER and traceability to the risk file.

Where a claim uses language such as “clinical decision support” or “clinical decision-making”, it declares G3 whether the drafter intended it or not.

Where the declared purpose implies a higher group than the actual output, one of the two is wrong, and the conflict is resolved before the classification is recorded.

Step 3: Users and oversight

Two populations are identified separately for each function: the direct user, who operates the function or reads its output, and the determined population, who receives care shaped by that output. Where the device sits behind an interface and a separate product renders its content, the direct user may be a system while the determined population is the public; the device authors lay-facing question content and permitted answer options even where it does not render them, and the IEC 62366-1 obligation for that content sits with the device regardless. The user population does not move the class under Annex IX, which is modality-based; it governs the evidence.

Population	Oversight	What it obliges
Trained non-clinical health advisor	Mandatory NHS Pathways training and competency. Disposition acted on within service protocol, with no routine clinical review of each output.	Training and competency is a stated risk control, recorded as an assumption in the intended purpose statement and not silently dropped when scope widens.
Registered clinician	Clinical judgement within a Clinical Assessment Service.	Anchoring on a displayed disposition or probability is an evidenced failure mode, so any displayed confidence value requires human factors evidence for this group. Introducing it creates a second intended purpose and its own risk analysis.
Lay, self-directed	A member of the public assessing their own symptoms via an approved digital channel. No training or clinical knowledge assumed.	IEC 62366-1 usability engineering including summative evaluation. Defined reading level and accessibility conformance. Comprehension evidence. Performance evidenced separately in self-directed mode: NHS Pathways content was validated for advisor-led telephone triage, and equivalence does not carry a change of use context.
Proxy or third party	A person reporting on behalf of another who cannot report for themselves.	Undefined in the intended purpose statement. Until it is defined, proxy reporting sits outside the declared intended purpose.
Clinical administrator	A clinician operating the administration interface: model configuration, content governance, threshold management, review of model behaviour.	Inside the device boundary, and its outputs configure clinical behaviour. Requires its own usability treatment, access control, audit trail and defined competency.

Step 4: Classification under Annex IX (UK MDR 2002)

Standalone software is an active medical device (Annex IX Section I paragraph 1.4), and regulation 7(1) of SI 2002/618 requires classification in accordance with Annex IX. Work through Rules 9 to 12 in turn and record why each is or is not engaged. A bare assertion of a class invites challenge; showing each rule was considered is what makes the conclusion survive audit.

Rule	Outline	Engaged by
Rule 9	Active therapeutic devices administering or exchanging energy. Class IIa, or Class IIb where energy may be administered or exchanged in a potentially hazardous way, and Class IIb for devices intended to control or monitor the performance of an active therapeutic Class IIb device.	Engaged by a function that administers energy, provides therapy, or controls or monitors an active therapeutic device.
Rule 10	Active devices for diagnosis. IIa where the device supplies energy absorbed by the body, images radiopharmaceutical distribution, or allows direct diagnosis or monitoring of vital physiological processes. IIb where it is specifically intended for monitoring vital physiological parameters whose variation could result in immediate danger to the patient, or emits ionising radiation for diagnostic and therapeutic interventional radiology.	Limbs 1 and 2 are not engaged: the device supplies no energy to the body and images nothing. The monitoring limb is not engaged and cannot be while the boundary condition against signal capture holds. The device receives reported observations and captures no physiological signal from an individual, so it has no vital physiological process to monitor. Rule 10 therefore engages on this platform through the direct diagnosis limb alone, at G3 and G4. The Class IIb limbs are unreachable on the same ground. Any proposal to capture a physiological signal reopens the monitoring limb and is assessed under Section 5 before design.
Rule 11	Active devices administering or removing medicinal products, body liquids or other substances. Class IIa, or Class IIb where administration or removal is potentially hazardous.	Engaged by a function that administers or removes a substance.
Implementing rule 2.3	Software driving or influencing the use of a device takes that device's class.	Engaged by a function that drives or influences a physical medical device. Section 5 carries this as a standing boundary.
Rule 12	All other active devices are Class I.	Engaged by any function reaching G1 or G2 where no other rule is engaged, and by a function that commits a clinical action without supplying a diagnosis. Where more than one rule applies, Annex IX Section II implementing rule 2.5 requires the strictest applicable rule, resulting in the higher classification, to govern.

Step 5: IEC 62304 software safety classification

Assess the severity of harm possible if the software fails or produces an incorrect output, before external risk controls are credited. Assign the highest applicable class.

Class	Definition	Application
A	No injury or damage to health is possible.	Not available to any function contributing to a disposition.
B	Non-serious injury is possible.	Available to peripheral functions that cannot affect the disposition, such as narrowing a pathway selection list, where the failure mode is bounded and evidenced.
C	Death or serious injury is possible.	The working position for every function inside the device boundary. Under-triage of a time-critical presentation can lead to death, there is no routine clinical review of each output, and the emergency screen is the control the safety argument rests on.

IEC 62304 class is independent of device class: the declared device is Class I under Annex IX and Class C under IEC 62304 at the same time. Lifecycle rigour is set by safety; the conformity route is set by classification. Under MD-QMS-WI-D002, decomposition inherits the parent class and a lower class for a sibling requires demonstrated separation on different machines or processors, so a function inside the boundary starts at Class C and argues down, with evidence, or not at all.

Step 5a: Non-device functions

A function classified G0 exits the determination at Step 1 and receives no Annex IX class and no IEC 62304 class. It does not exit the rubric. It remains subject to:

- Boundary conditions, stated in PT-2.1 as testable design constraints, defining what the function must not do.
- A separation test under 4.2, completed and evidenced.
- A change-control trigger: any proposed change is assessed against those boundary conditions before implementation.

4. Gatekeeping by function

4.1 What the gate is for

A function claims its own classification. No function is recorded at a class lower than a function it shares infrastructure, data or a release train with until the separation test is complete and the contamination controls are in place. The gate applies at three points, and the second is the one most often skipped.

- At initial classification of the function.
- At the design stage gate, before development begins on any change to any function. A change to a non-device function is in scope. A change to the platform is in scope.
- At every change control assessment under MD-QMS-SOP-D007.

4.2 The separation test

Complete for every function claiming a class lower than any function it neighbours. All three criteria must be met. A criterion that cannot be met is recorded as not met, and the function takes the higher class.

#	Criterion	Evidence required
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S1	Boundary. The function has its own intended purpose statement, stating what it does and what it specifically does not do, and the boundary to every higher-group neighbour is identified and specified, not merely described.	The statement in PT-2.1, approved. Interface specification enumerating every field crossing the boundary in each direction.
S2	Independence. No output of a lower-group function is relied on by a higher-group function as a clinical input or control, other than through the specified interface; and no output of the lower-group function carries clinical meaning about an individual, directly or by ordering, filtering, highlighting or omission.	Dependency map, and code-level verification that no other path exists. Output schema review, and the PT-2.1 boundary conditions verified against the build.
S3	Separability and safety. The function can be modified, versioned and released without modifying a higher-group function, and the reverse; and the whole combination, including the connection system, is safe and does not impair the specified performance of the regulated functions.	Build and release configuration; deployment topology. Integration verification; risk analysis of the interface itself, not only of the functions either side.

4.3 Contamination controls

Standing controls on the platform, independent of any individual change.

Applies to	Statement
C1 Non-device functions	No ranking, sorting, filtering, flagging, branching or highlighting of individuals on any basis derived from clinical content, symptom responses, disposition or confidence value. No language implying clinical significance in any output or label: no alert, concern, risk, priority, deterioration, insight or equivalent. Every emitted field of a conduit function traceable to an identical received field, with no derived field or selective omission.
C2 Platform configuration	No clinical parameter held or settable in the platform configuration surface. Thresholds, content versions and disposition mappings are held only in the clinical administration interface inside the device boundary.
C3 All functions inside the device boundary	No output names, implies, ranks or attaches a likelihood to a clinical condition, including through SNOMED CT concept selection, since a coding change looks like a data change rather than a scope change. Any confidence, probability or score surfaced expresses confidence in a route to care; nothing expresses the likelihood of a condition or the severity of a presentation.
C4 All functions	Clinical parameter changes route through MD-QMS-SOP-D007 and, where design or intended use is touched, back to MD-QMS-SOP-D001 Gateway 0 via a Change Requirements Specification.

4.4 The determination record

Every determination is recorded on a determination record, one per function. A function holds no supported classification claim until its record is complete and approved, and the record is the document Section 6 requires to be reissued on change.

The record carries: the function name and the version of the build it was assessed against; the Step 1 qualification outcome; the assigned group G0 to G4; the Annex IX rule engaged and the resulting class,

with the reason each other rule was considered and not engaged; the IEC 62304 class and the basis for it; the result against each separation criterion S1 to S3 with the evidence cited; the boundary conditions the function is held to; open items with owners; and the assessor, the approver and the date. The record is held as a controlled form; the MD-QMS form reference is to be assigned by the RA office.

5. Escalation triggers

Class I is a position on a ladder, and holding it is a design discipline with a cost, exactly as moving off it is. A proposal is assessed against the transition it risks, not a general sense of risk.

From	To	Trigger
G0, non-device	G1 or above, device	Ranking, sorting, filtering, flagging or prioritising individuals on any clinically derived basis; adding, deriving or inferring a field not present in the input; summarising, abstracting or selectively omitting clinical content; presenting an operational metric with clinical meaning at the level of an individual; holding or exposing a clinical parameter in the platform configuration surface.
Class I, Rule 12 (G1, G2)	Class IIa, Rule 10 (G3, G4)	Generating, retaining, exposing or relying on any condition-level information; capturing a physiological signal directly from the individual rather than receiving a reported observation, or analysing an image rather than forwarding it; delivering a clinical assurance to an individual that carries condition-level information.
Class IIa, Rule 10 (G3)	Class IIa, Rule 10 (G4)	Removing, bypassing or degrading the independent judgement the G3 position rests on. Presenting condition-level content to a lay user; presenting it to a trained non-clinical user with no routine clinical review; wording it so that a reader is not expected to second-guess it; or reducing the reader's role to confirmation. The class does not change, so nothing in the conformity route flags it. Reassess the clinical evidence, the usability file and the risk file, and reissue the determination record.

Rule 10's Class IIb limbs are not reachable from any position this platform occupies. They require monitoring of a vital physiological parameter, or emission of ionising radiation, and neither is within technical scope. The transition to watch is the one above: capture of a physiological signal is what would first put the monitoring limb into play, and it is already a Class I to Class IIa trigger.

Class I holds for a function at G1 or G2 provided:

- No capture or interpretation of a physiological signal from the individual, from a connected sensor, a wearable, an uploaded image analysed by the device, or any other telemetry. This is the boundary condition that holds Rule 10's monitoring limb, and with it the Class IIb limbs, structurally out of reach.
- No candidate condition generated, retained, exposed or relied upon. Contamination control C3 at 4.3 applies in full.
- No confidence, probability or score expressing the likelihood of a condition or the severity of a presentation.

- No condition-level statement, finding or negative assurance about an individual, whether delivered to that individual, delivered to a reader, or held internally and used to select, rank or derive any output. Supplying one is the move from G2 to G3. Supplying it as the operative conclusion is the move to G4.
- No commitment of a clinical action by the device itself, as distinct from returning a recommendation for a service to act on.
- No extension of the assessment beyond a single contact episode, by record-derived input, retained history, or model behaviour that changes with previous outcomes.
- No administration or removal of substances, and no supply or exchange of energy.
- No control of, or influence over, another active medical device.

6. Change control

Every change is assessed against the determination record at 4.4 for the function it touches. Where a change touches more than one function, the highest required action applies and every affected record is reissued.

Change type	Required action	Approval
Wording of any intended purpose statement	Reassess Step 4, and Step 2 where the wording describes an output. Reissue the record.	RA, with the Management Representative
New or altered output from any function	Full determination across the steps above, plus the separation test. Assess against the escalation triggers in Section 5.	RA and MDP where the class changes
New input type	Full determination. Specifically test the Rule 10 monitoring limb and cross-episode personalisation.	RA; MDP where Rule 10 is engaged
New user population or deployment channel	Reassess Step 3 and the evidence obligations. Reissue the usability plan.	RA, with the CSO
Change to a threshold, including early stopping	Not a classification change but a clinical safety change. Route through MD-QMS-SOP-D007, and to MD-QMS-SOP-D001 Gateway 0 where declared behaviour alters. Update the risk file and the clinical safety case.	CSO and RA
Model retraining or parameter update	Design change under ISO 13485 7.3.9. Validate and release under the QMS. Assess for significance. Confirm no new output type has emerged from the retrained model, which is the failure mode a retraining process is least likely to look for.	Per MD-QMS-SOP-D007
Change to a non-device function	Assess against the contamination controls and the function's boundary conditions. Reissue the record if any boundary condition is affected.	RA

Change to a module boundary or interface	Repeat the separation test at 4.2 in full. A boundary change invalidates every determination that relied on it.	RA and engineering lead
Introduction of a function recorded as potential future functionality	Full determination before design work begins. Approved revision of PT-2.1 before implementation. Approved Body engagement where Class IIa is entered.	MDP

7. The gate applied

Six proposals, each of which arrives looking like an improvement rather than a scope change.

Proposal	Outcome
Sort the advisor worklist so low-confidence assessments appear first.	Fails contamination control C1. Prioritisation on a clinical basis converts a non-device function from G0 into a device function. If the service needs it, it belongs inside the boundary with its own determination.
Show the clinician the top three conditions considered, greyed out, marked as not a diagnosis.	Fails contamination control C3 and the condition-level descope recorded in PT-2.1. G3 on its face: the output supplies candidate conditions to a reader. Class IIa, Approved Body, revised PT-2.1 before anything is built.
Use the internal condition distribution to pick a better service from the DoS, showing nothing.	Fails contamination control C3. Information the device acts on is information the device provides. The case the descope was written to exclude.
Accept a pulse oximeter reading over Bluetooth rather than typed entry.	Class I to Class IIa: capturing a physiological signal rather than receiving a reported observation. Content and outputs unchanged, which is what makes it feel like an integration task. A Rule 10 determination and an Approved Body conversation.
Let the model use the patient's previous 111 contact to skip answered questions.	Cross-episode personalisation. Record-derived input, a dependency on record accuracy the device cannot verify, and an assessment outside a single contact episode. Full determination and a different intended purpose statement.
Add a timeline view of how answers developed during the call.	Passes, provided separation criterion S2 and contamination control C1 hold: no highlighting, no ordering by clinical significance, no omission, no language implying clinical meaning. A legitimate G0 function, one careless design decision from being a G3 one.

8. Operational discipline

- Each intended purpose statement is a single, self-contained, version-controlled text, reproduced verbatim wherever it appears: technical documentation, Declaration of Conformity, labelling, instructions for use, MHRA registration. Distributing it across sections in variant prose allows scope to widen in one place without the classification being retested.

- Negative indications appear on the labelling and in the user-facing product, not only in internal assessments.
- Classification is recorded per function before development starts, and confirmed or updated at every change control review.
- The intended purpose statements and the DCB0129 clinical safety case describe the same scope.
- Any function approaching an escalation trigger, particularly sensor integration or condition-level inference, is escalated before implementation rather than at review.
- Marketing, service communications and user-facing content are reviewed against the registered intended purpose.

Sources and applicable framework

Reference	Title and status
UK MDR 2002	The Medical Devices Regulations 2002, SI 2002/618, as amended. Part II applies the Annex IX criteria of Directive 93/42/EEC in Great Britain. Primary source.
MHRA software guidance	Medical device stand-alone software including apps (including IVDMDs), MHRA guidance 0951, v1.10f, July 2023. Appendix 1 Symptom checkers is the most directly applicable UK source.
MHRA intended purpose guidance	Crafting an intended purpose in the context of software as a medical device, 2023. Source of the working SaMD definition applied at Step 1.
MHRA AVT guidance	Ambient voice technology-enabled products, 29 July 2026. Holds that intended purpose is not determined by the inclusion of any particular technology, that disclaimers cannot override claims made elsewhere, and that expansion of functionality on a flexible underlying technology triggers reassessment. Supports the method in Sections 3 and 5.
MEDDEV 2.1/6	Qualification and classification of standalone software. Applied for qualification.
MEDDEV 2.4/1 Rev 9	Classification of medical devices, June 2010. Applied for the active-device framework and vital physiological processes. Declines to address standalone software at section 3.2, so cannot carry the argument alone.
MDCG 2019-11 Rev 1	Qualification and classification of software, rev 1, June 2025. Section 7 is the authority for the function architecture.
IMDRF/SaMD WG/N12:2014	Framework for risk categorization.
IMDRF/SaMD WG/N81:2025	Characterization considerations for medical device software and software-specific risk, 27 January 2025.
IEC 62304:2006+A1:2015	Medical device software life cycle processes.
BS EN ISO 14971:2019+A11:2021	Risk management.
BS EN 62366-1:2015+A1:2020	Usability engineering.
BS/AAMI 34971:2023	ISO 14971 applied to machine learning in AI.
BS EN 82304-1:2017	Health software product safety. Applies to the product as a whole, non-device functions included, independently of device status. Its application is an open item.
DCB0129 / DCB0160	Clinical risk management in the manufacture, and in the deployment and use, of health IT systems. Runs alongside the device framework, not in place of it.

SI 2024/1368	The Medical Devices (Post-Market Surveillance Requirements) Regulations 2024, in force 16 June 2025. Applies in full to a Class I self-declared device: post-market surveillance plan, trend reporting and periodic review. Drives NHSP-10.4.
Draft 2026 Regulations	Draft Medical Devices (Amendment) Regulations 2026 (WTO notification 8 May 2026; not yet in force, adoption anticipated December 2026). Introduces cybersecurity, Predetermined Change Control Plan and misleading-claims obligations. Software risk classification is not addressed in the published draft, deferred pending the National Commission on the Regulation of AI in Healthcare, launched September 2025. Regulatory horizon item, not relied on for any recorded classification. No impact can be confirmed until the instrument is made and its commencement and transitional dates are known; the position is reviewed on publication.
MD-QMS	NHS England MD-QMS, branches A to F. SOP-D001, D002, D007, D011, D017 and WI-D002 apply directly.